NORTHGATE MULTISPECIALITY HOSPITAL

22/B, Placeholder Marg, Sample Vihar, New Delhi - 110000 | Ph: 011-0000 0000 | CIN: U00000DL0000PTC000000 (illustrative)
NABH Accredited (sample) | www.northgate.example

DISCHARGE SUMMARY

Patient Name:	**Mr. ARJUN MEHTA**	IPD No.:	**IP/2026/045512**
Age / Sex:	**58 Years / Male**	UHID:	**NG-0098731**
Address:	**H-00, Sample Colony, New Delhi - 110000**	Mobile:	**+91 00000 00000**
Date of Admission:	**14/10/2026 06:20 PM (via Emergency)**	Ward / Bed:	**Ward 5B / 214**
Date of Discharge:	**19/10/2026 11:00 AM**	Length of Stay:	**5 days**
Treating Consultant:	**Dr. Neha Example, MD (Gen. Medicine), DM (Endocrinology)**	Payer / TPA:	**Cash / Insurance (sample ID: SI-000000)**

DIAGNOSIS

Final diagnosis: Uncontrolled Type 2 Diabetes Mellitus with symptomatic hyperglycaemia and ketosis (without acidosis) *(ICD-10: E11.65)*
Secondary diagnoses: Essential hypertension *(I10)*; Dyslipidaemia *(E78.5)*; Early diabetic nephropathy (microalbuminuria) *(E11.21)*

PRESENTING COMPLAINTS & BRIEF HISTORY

58-year-old male, known T2DM for 11 years on oral agents (non-compliant for ~1 month) and hypertension for 6 years, presented with polyuria, polydipsia, fatigue and unintentional weight loss of 4 kg over 3 weeks, with 2 days of nausea and generalised weakness. No fever, chest pain or breathlessness. No history of CAD or stroke. Family history: father diabetic.

On examination: Conscious, oriented. Mild dehydration. Pulse 104/min, BP 148/92 mmHg, RR 20/min, Temp 98.4 F, SpO2 98% RA, BMI 27.8 kg/m2. Systemic examination: CVS - S1 S2 normal; RS - clear; P/A - soft, non-tender; CNS - no focal deficit.

INVESTIGATIONS

Investigation	On Admission	At Discharge	Reference
Random blood glucose (mg/dL)	486	142	70 - 140
HbA1c (%)	11.2	-	< 5.7 (non-diabetic)
Urine ketones	2+ (Positive)	Negative	Negative
Arterial pH / HCO3 (mEq/L)	7.36 / 20	7.40 / 24	7.35-7.45 / 22-26
Serum creatinine (mg/dL)	1.3	1.0	0.7 - 1.3
Serum sodium / potassium (mEq/L)	131 / 4.8	138 / 4.1	135-145 / 3.5-5.0
Total cholesterol / LDL (mg/dL)	238 / 152	-	< 200 / < 100
Urine albumin:creatinine ratio (mg/g)	84	-	< 30
ECG	Sinus tachycardia, no ST-T changes	-	-
USG Abdomen	Grade I fatty liver; kidneys normal	-	-

COURSE IN HOSPITAL

Patient was admitted, rehydrated with IV normal saline and started on a basal-bolus subcutaneous insulin regimen with 6-point glucose monitoring. Ketosis resolved within 36 hours; electrolytes remained within acceptable limits and no acidosis developed. Oral agents were reviewed - metformin resumed after renal function was confirmed adequate. Blood pressure was optimised with telmisartan, and atorvastatin was started for dyslipidaemia. Diabetes educator and dietitian counselling were provided, including insulin self-administration training. Ophthalmology review: no diabetic retinopathy on dilated fundus exam. Patient remained haemodynamically stable and was discharged in a satisfactory condition.

SYNTHETIC SAMPLE - fictional patient, hospital and clinicians. Not a medical record. For software testing only.

TREATMENT GIVEN

IV fluids (0.9% NaCl); Inj. human insulin (regular) SC as per sliding scale; Inj. insulin glargine; Inj. pantoprazole 40 mg IV OD; antiemetics PRN.

CONDITION AT DISCHARGE

Stable, afebrile, ambulatory, tolerating oral diet. Pulse 78/min, BP 128/80 mmHg, SpO2 98% RA. Fasting glucose on day of discharge: 118 mg/dL.

DISCHARGE MEDICATIONS

#	Drug	Dose	Route	Frequency	Duration
1	Tab. Metformin SR 500 mg	1 tablet	Oral	Twice daily after meals	Continue
2	Inj. Insulin Glargine 100 IU/mL	18 units	SC	Once daily at bedtime	Continue; titrate
3	Inj. Insulin Aspart	6 units	SC	Thrice daily, 15 min before meals	Continue; titrate
4	Tab. Telmisartan 40 mg	1 tablet	Oral	Once daily, morning	Continue
5	Tab. Atorvastatin 20 mg	1 tablet	Oral	Once daily, night	Continue
6	Tab. Pantoprazole 40 mg	1 tablet	Oral	Once daily before breakfast	7 days

ADVICE / DIET / ACTIVITY

1. Diabetic diet (approx. 1,800 kcal/day), low salt; avoid sugary drinks and refined carbohydrates.
2. Walk 30 minutes daily; avoid strenuous exercise until review.
3. Check blood glucose fasting and 2 hours after dinner; keep a log and bring it to every visit.
4. Hypoglycaemia precautions explained: keep glucose tablets handy; seek help if glucose < 70 mg/dL with symptoms.
5. Daily foot inspection; wear protective footwear.

Return to Emergency if: persistent vomiting, abdominal pain, drowsiness, rapid breathing, glucose > 400 mg/dL or < 60 mg/dL.

FOLLOW-UP

Endocrinology OPD (Dr. Neha Example) on **26/10/2026** between 10:00 AM - 1:00 PM with glucose log. Repeat HbA1c, lipid profile and kidney function after 3 months. Annual retinal and foot screening. For emergencies call 011-0000 0000 (24x7).

Dr. Neha Example
MD, DM (Endocrinology)
Senior Consultant
Reg. No. DMC-000000
Signature: _____

Patient / Attendant
Name: _____
Relation: _____
Signature: _____

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